

CLINICALLY INTELLIGENT

ISSUE 04 · COMPLETE PACK · APRIL 2026

THREE TOOLS, ONE WORKFLOW

Three Tools. Three Lanes. *One Workflow.*

All three prompts and the carer guidance template in one place. Find. Decide. Explain. Build the library.

CONTENTS

Inside this pack.

01	Read this first	p. 03
	<i>Information governance and what this pack is</i>	
02	The three lanes	p. 04
	<i>Find. Decide. Explain. Match the lane to the question.</i>	
03	Prompt 01 — The Roleplay	p. 05
	<i>ChatGPT · Pressure-test the conversation</i>	
04	Prompt 02 — The Operational Research	p. 07
	<i>Perplexity · Find what is currently true</i>	
05	Prompt 03 — The Documentation	p. 09
	<i>Claude · Bring it together and explain it clearly</i>	
06	Worked example	p. 11
	<i>Cognitive rehabilitation after TBI · Composite scenario</i>	
07	The Carer Guidance Template	p. 12
	<i>The scaffold Prompt 03 produces. Use it as a starting point.</i>	
08	Pre-flight checklist & distribution rules	p. 20
	<i>Thirty seconds before every workflow.</i>	

PART 01

Read this *first*.

Before you use any of the prompts in this pack, the rule that runs underneath all of them.

INFORMATION GOVERNANCE

This pack builds library entries from composite scenarios — patient types, not real patients. When you apply a library entry to a real patient, do so within your Trust's documentation systems. Any version with real patient information becomes a clinical record and your Trust's IG and records policies apply. If you are not sure what those are, your IG lead is the person to ask.

WHAT THIS PACK IS

This is the full Issue 04 toolkit. Three prompts, one worked example, and the scaffold the workflow produces — all in one document so you can read it once and refer back to it whenever you need to.

Each prompt is standalone. You do not need to read them in order. Skip to the one you need. Use the contents page above.

If you are new to this approach, read Part 02 first. It explains why three tools and not one, and how the lanes connect.

PART 02

The three *lanes*.

Most clinical tasks need more than one tool because most clinical tasks need more than one kind of thinking.

Match the lane to the question. Find / Decide / Explain. The tools are different. The thinking is different. The mistake most clinicians make is using one tool for everything because it is the one they have heard of.

Perplexity	<i>Find</i>	What is currently true. Citations, sources, evidence.
ChatGPT	<i>Decide</i>	What would a person actually ask me. Roleplay, pressure-testing, surfacing.
Claude	<i>Explain</i>	Can you bring this together and explain it clearly. Documentation, structured output.

Each lane has a prompt. Each prompt has a job. Together they produce a library entry — a reusable scaffold of guidance for the conversations you have most often. Not a patient record. A scaffold.

PART 03 · PROMPT 01

The Roleplay Prompt.

Pressure-test the conversation before you have it for real.

USE WITH
ChatGPT

THE PROMPT

COPY · REPLACE BRACKETS · RUN

You are a [family member or patient] of someone facing [condition or situation]. You have no medical background. You are tired and worried. I am the [health professional] involved in their care. Ask me the questions you would actually have. Tell me when I use words you do not understand. Tell me when an instruction is not clear enough to follow at home. Push back if my answers are too clinical. Keep going until you feel you understand what is happening and what you need to do.

WHEN TO USE THIS

Before any high-stakes patient or carer conversation where what gets remembered matters more than what gets said.

- Discharge explanations to a family who will not see you again until next week.
- Carer instructions that have to survive the school run, the night shift, and the bad day.
- Self-management plans that work in clinic and fall apart at home.
- Anything you have explained ten times and still feel you have not landed properly.

HOW IT WORKS

ChatGPT plays the person on the other side of the conversation. Tired. Worried. No medical background. It asks the questions a real family member would ask, flags the words it does not understand, and pushes back when your answers are too clinical.

It does not invent the questions. It surfaces the ones you have stopped hearing because you have answered them a hundred times.

WHAT GOOD LOOKS LIKE

- The roleplay surfaces a question you had not thought to answer.
- It catches a phrase you use without thinking that a layperson does not understand.
- It exposes the gap between what you say and what gets remembered.
- You finish the session with a clearer version of the explanation than the one you started with.

COMMON MISTAKES

Letting it stay polite.

If the AI is not pushing back, it is not doing the job. Tell it to be more anxious. More tired. More confused. The point of the exercise is friction.

Stopping when it gets uncomfortable.

The line that stops you is the one you needed to hear. Keep going.

Treating the output as the answer.

The roleplay is the diagnostic, not the script. What you do with what it surfaced is the actual work.

PART 04 · PROMPT 02

The Operational *Research* Prompt.

Find what is currently true. Translate it into NHS operational reality.

USE WITH

Perplexity**THE PROMPT**

COPY · REPLACE BRACKETS · RUN

Find the most relevant studies on **[topic]**. Then translate the findings into NHS operational impact. For each insight include:

- what the study found
- what it means in practice for NHS services
- impact on cost, staffing, or patient flow
- whether it is realistically implementable

Include citations.

WHEN TO USE THIS

When you need current evidence framed for the way services actually run, not the way papers are written.

- Guideline updates. NICE, NHS England, MHRA, Royal College position statements.
- Service design questions. Pathway redesign, capacity modelling, workforce planning.
- Business cases that need evidence and operational framing in the same paragraph.
- CPD where you need to be current and you do not have a journal subscription.

HOW IT WORKS

Perplexity searches the live web, returns sources with citations, and lets you ask follow-ups in the same thread. The prompt below pushes it past summary and into operational translation.

Reminder. Perplexity surfaces broken links and outdated guidance just like any other AI. Click every link before you cite anything.

WHAT GOOD LOOKS LIKE

- Sources cited and dated. You have clicked every link before you cited anything.
- Operational framing that names the trade-off, not just the finding.
- A short list of implementable actions, not a literature review.
- You finish with three things you can act on this week.

COMMON MISTAKES

Asking too broad.

"Find studies on falls" gets you a tutorial. "Find the most relevant studies on falls prevention in community-dwelling adults over 75 in the last three years, with NHS-relevant operational framing" gets you something useful.

Skipping the verification.

Perplexity will cite a paper that does not say what it claims it says. Click. Read. Verify.

Treating it like Google.

Perplexity rewards specificity and follow-up questions. The first answer is rarely the best answer. Push it for what is missing, what is outdated, what contradicts.

PART 05 · PROMPT 03

The Documentation *Prompt*.

Bring the questions and the evidence together. Explain it clearly.

USE WITH

Claude

THE PROMPT · TWO PARTS

COPY BOTH · REPLACE BRACKETS · RUN

PART 1 · THE INSTRUCTION

Act as a clinical documentation assistant helping a [health professional] build a reusable [type of intervention] guidance template. I will give you a composite patient scenario and the [strategies / exercises / techniques] I have prescribed. Produce the output as a document laid out simply and clearly so a patient or carer can read it without effort. Use clear headings, short paragraphs, plain English at reading age eleven, and a warm tone written directly to the patient and the family member supporting them. Explain each [strategy / exercise / technique] using established clinical knowledge. Do not invent patient specific details. If something has not been provided write 'to be personalised by clinician' in that field. Scenario and [strategies / exercises / techniques]: [PASTE HERE]

PART 2 · THE TEMPLATE SCAFFOLD

Template:

About this guidance.

- What these [strategies / exercises / techniques] are for.
- Why your clinician has chosen them for you.
- What they should help with day to day.

[Strategy / Exercise / Technique] one through four.

- What this is.
- When to use it.
- How to use it.
- How your family or carer can help.
- What good looks like.
- If it is not working.

Building these into your day.

- How to fit these into normal life without it feeling like extra work.
- How to start small.
- What to do on a hard day.

When to get in touch.

- Contact your team if.
- Contact your GP if.
- Call 111 if.
- Call 999 if.

Questions for your next visit. (Space for the patient and family.)

Notes from home. (Space for the family or carer.)

WHEN TO USE THIS

When you have done the listening and the research and you need to produce written guidance that survives the journey from clinic to kitchen table.

- Carer guides for families managing a relative at home.
- Self-management plans that need to be readable on a hard day.
- Reusable templates for the conversations you have most often.
- Anything where the patient or family has to do the work and you cannot be there.

HOW IT WORKS

Claude takes the role, the context, and the information in that order, and produces structured written guidance at reading age eleven. The prompt above is the full scaffold. Replace the bracketed sections with the intervention type and the composite scenario.

This prompt builds library entries from composite scenarios. The output is reusable guidance. Personalisation for a real patient happens inside your Trust's documentation systems.

WHAT GOOD LOOKS LIKE

- Output that reads like it was written for a person, not a patient.
- Reading age eleven, verified by reading it aloud once before you give it to anyone.
- Every strategy follows the same six prompts. Families find consistency easier to retain.
- Nothing fabricated. Every piece of patient-specific information either comes from your assessment or is flagged for personalisation.

COMMON MISTAKES

Pasting real patient information into the prompt.

Use composite scenarios. A composite is a patient type, not a person. The output is just as good. The IG risk is not.

Accepting the first draft.

Read it aloud. Where you stumble, simplify. Where it feels generic, sharpen. The first draft is a starting point, not the deliverable.

Forgetting it is a library entry, not a record.

The output is a reusable scaffold. It becomes a clinical record only when you personalise it for a real patient inside your Trust's documentation systems. Keep that distinction clear in your own head and in your filing.

PART 06

Worked example.

Cognitive rehabilitation after TBI. A composite scenario, not a real patient. Use it to see how the three lanes connect.

STEP 1 · CHATGPT — THE ROLEPLAY

Composite scenario: a spouse in their late sixties, no medical background, supporting a partner returning home after a fall-related traumatic brain injury. Tired. Worried. Wanting to help.

Prompt 01 was used to roleplay the spouse. The line that came back:

"I want to help them, I really do. But when we are actually at home, in the middle of the day, I do not always know what I am supposed to do."

That is every carer you have ever met. ChatGPT did not invent it. It surfaced it. Now you know what the written guidance has to answer.

STEP 2 · PERPLEXITY — THE EVIDENCE

Prompt 02 was used to pull current UK guidance on cognitive rehabilitation after fall-related TBI. Eighteen sources surfaced including NICE NG211 and QS74. Compensatory strategies. Environmental adaptations. Carer involvement. Progression principles.

Two things you now have that ChatGPT could not give you on its own:

- The questions families actually ask.
- The evidence base you need to answer them properly.

STEP 3 · CLAUDE — THE DOCUMENT

Prompt 03 was used with three things in order: the role (clinical documentation assistant), the context (carer questions from ChatGPT plus evidence summary from Perplexity), and the information (composite scenario plus the four strategies prescribed).

Output: a draft carer guide written for both the patient and the spouse. Reading age eleven. Warm tone. Structured for retention.

That draft becomes a library entry. Reusable. Not a real patient record. The scaffold the workflow produces is the next part of this pack. Use it.

PART 07

The Carer Guidance *Template.*

The scaffold Prompt 03 produces. Use it as the starting point for any carer-facing guidance you write.

REMINDER

This template is a clinician-facing scaffold. It is not a patient-facing leaflet. Personalise placeholders for the individual patient you have assessed, and route any material intended for general distribution through your Trust's patient information governance process before sharing.

HOW TO USE THIS TEMPLATE

- Use composite scenarios when drafting with AI. Do not paste real patient details into ChatGPT, Perplexity, or Claude.
- Personalise inside your Trust's documentation systems where the real patient context lives.
- Read the full document aloud once before you give it to the family. If you stumble on a sentence, simplify it.
- Date and version every copy you give out. Keep a record of what was given to whom.

PATIENT DETAILS

FOR

Patient name or 'the person you are supporting'

PREPARED BY

Your name, role, and team

DATE

Date this guidance was produced

REVIEW BY

Date this guidance should be reviewed

ABOUT THIS GUIDANCE

What this is for.

One or two sentences. What the strategies in this document are designed to help with. Plain English.

Why your clinician has chosen this for you.

The clinical reasoning, translated. Avoid jargon. Why these strategies, why now, why this person.

What this should help with day to day.

The practical difference the family should expect to see if it is working. Be specific. Avoid clinical outcome language.

STRATEGIES

Each strategy follows the same six prompts. Keep the structure even if the content is short. Families find it easier to follow when every strategy looks the same on the page.

STRATEGY 01

[Name of strategy]

What this is.

Plain English explanation. One or two sentences. Reading age 11.

When to use it.

The situations where this strategy applies. Be specific.

How to use it.

Step-by-step. Numbered if there is a sequence. Short sentences.

How your family or carer can help.

What the person supporting them can do. Practical actions.

What good looks like.

How they will know it is working. Observable. Not clinical.

If it is not working.

What to try first. When to stop. When to get in touch.

STRATEGY 02

[Name of strategy]

What this is.

Plain English explanation. One or two sentences. Reading age 11.

When to use it.

The situations where this strategy applies. Be specific.

How to use it.

Step-by-step. Numbered if there is a sequence. Short sentences.

How your family or carer can help.

What the person supporting them can do. Practical actions.

What good looks like.

How they will know it is working. Observable. Not clinical.

If it is not working.

What to try first. When to stop. When to get in touch.

STRATEGY 03

*[Name of strategy]***What this is.**

Plain English explanation. One or two sentences. Reading age 11.

When to use it.

The situations where this strategy applies. Be specific.

How to use it.

Step-by-step. Numbered if there is a sequence. Short sentences.

How your family or carer can help.

What the person supporting them can do. Practical actions.

What good looks like.

How they will know it is working. Observable. Not clinical.

If it is not working.

What to try first. When to stop. When to get in touch.

STRATEGY 04

[Name of strategy]

What this is.

Plain English explanation. One or two sentences. Reading age 11.

When to use it.

The situations where this strategy applies. Be specific.

How to use it.

Step-by-step. Numbered if there is a sequence. Short sentences.

How your family or carer can help.

What the person supporting them can do. Practical actions.

What good looks like.

How they will know it is working. Observable. Not clinical.

If it is not working.

What to try first. When to stop. When to get in touch.

BUILDING THIS INTO YOUR DAY**How to fit this into normal life.**

How the strategies fit around existing routines. Meals, washing, getting dressed, the school run, work. Avoid asking the family to add a new thing.

How to start small.

The smallest version of the strategy that still counts. The thing they can do on the first day without preparation.

What to do on a hard day.

Permission to scale back. What still counts as a good day. What to drop and what to hold.

WHEN TO GET IN TOUCH**CONTACT YOUR TEAM IF**

Specific signs that the family should escalate to the named clinician or service.

CONTACT YOUR GP IF

Signs that warrant primary care review but are not urgent.

CALL 111 IF

Signs that need urgent advice but are not life-threatening.

CALL 999 IF

Red flags. Be specific. The family should not have to interpret these.

QUESTIONS FOR YOUR NEXT VISIT

Use this space to write down anything you want to ask the next time we meet.

NOTES FROM HOME

Use this space to write down what is working, what is not, and any changes you have noticed. Bring it with you next time we meet.

PART 08

Pre-flight checklist.

Run through this before every workflow. Thirty seconds. The difference between a clean library entry and a clinical record breach.

- ☐ Have I converted the patient in front of me into a composite scenario?
- ☐ Have I removed every identifying combination — name, condition with age and area, anything that would identify them to someone with reasonable knowledge of my service?
- ☐ Am I using the right lane for the right question? Find / Decide / Explain.
- ☐ If I am citing evidence, have I clicked the link to verify it exists and says what the tool claims it says?
- ☐ If the output is going anywhere beyond a single patient I have assessed, do I know which IG and patient information governance route applies?
- ☐ Have I dated and versioned the output?

DISTRIBUTION RULES

Individual patient guidance you have assessed and signed off.

Sits within your normal professional scope. Personalise inside Trust documentation systems. Date and version every copy.

General distribution material — leaflets, multi-patient handouts, anything Trust-branded.

Different territory. Take the draft to whoever governs patient information in your Trust before it goes anywhere. The route exists. Use it.

Consumer AI tools and IG.

ChatGPT, Perplexity, and Claude are consumer products. They have not been through NHS procurement or been assessed against DCB0129 or DCB0160 — the clinical risk management standards NHS health IT systems are assessed against. Until your IG lead tells you otherwise, keep real patient context out of the tools entirely.

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